

## SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

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| <b>Session:</b> Revision Mock Set 05 | <b>Duration:</b> 3 hours                              |
| <b>Specialty:</b> Psychotherapy      | <b>Coefficient:</b> 04                                |
| <b>Eliminatory mark:</b> < 8/20      | <b>Training reference:</b> VTI-AEDA examination style |

## COMPREHENSIVE VOCATIONAL PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 05

*Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.*

### How to use this marking guide

For revision, first attempt the paper without notes. Then compare not only the answer letter but the clinical cue that makes the answer correct. For legal/ethical items, the guide follows the concepts emphasized by the supplied course/examination materials; in real practice, applicable Cameroon law, institutional policy and the current professional code must also be checked.

### PART ONE: PSYCHOPATHOLOGY - ANSWERS

1. During assessment, Hassan reports a problematic pattern of substance use causing clinically significant impairment or distress, including loss of control, risky use, social impairment or tolerance/withdrawal. Which of the following is the BEST match?

- A. Substance use disorder
- B. Schizophrenia
- C. Major depressive disorder
- D. Binge-eating disorder

**Answer: A - Substance use disorder.** Substance use disorder is diagnosed from a maladaptive pattern and consequences, not simply from having used a substance.

**Memory aid:** 4C idea: Control loss, Consequences, Compulsion/craving, Continued use.

2. During assessment, Kemi reports initial euphoria followed by drowsiness or apathy, with slowed functioning and often constricted pupils. Which of the following is the BEST match?

- A. Schizoid personality disorder
- B. Panic disorder
- C. Opioid intoxication
- D. Substance use disorder

**Answer: C - Opioid intoxication.** Opioids typically produce analgesia, euphoria followed by sedation and physiologic slowing.

**Memory aid:** OPIOID = Often Pinpoint pupils, Inward/sedated, Opioid drowsiness.

3. Yvette, a 25-year-old university student, presents with euphoria, increased energy and confidence, decreased need for sleep, agitation and autonomic activation. Which diagnosis or clinical concept best fits this presentation?

- A. Anorexia nervosa, restricting type
- B. Substance use disorder
- C. Schizophrenia
- D. Stimulant intoxication

**Answer: D - Stimulant intoxication.** Stimulants activate rather than sedate; cocaine and amphetamines commonly produce energy, alertness and agitation.

**Memory aid:** STIMULANT = STIMulates.

4. A therapist notes that Quentin has pervasive instability in relationships, self-image and affect, impulsivity, abandonment fears and recurrent self-harm or suicidal behavior. Which condition should be considered first from the options below?

- A. Autism spectrum disorder
- B. Schizophrenia

**C. Borderline personality disorder**

D. Major depressive disorder

**Answer: C - Borderline personality disorder.** Borderline personality disorder is marked by instability and intense reactivity across relationships, identity, mood and behavior.

**Memory aid:** BORDER: Bonds unstable, Overreactive affect, Risky acts, Desertion fear, Empty/identity shifts, Recurrent self-harm.

5. Rose, a 55-year-old sales representative, presents with perfectionism, orderliness, rigidity and control that interfere with flexibility and task completion. Which diagnosis or clinical concept best fits this presentation?

A. Post-traumatic stress disorder (PTSD)

B. Stimulant intoxication

C. Substance use disorder

**D. Obsessive-compulsive personality disorder (OCPD)**

**Answer: D - Obsessive-compulsive personality disorder (OCPD).** OCPD is a personality pattern of perfectionism and control; unlike OCD, true obsessions and compulsions are not required.

**Memory aid:** OCPD = Order, Control, Perfectionism, Duty.

6. Quentin, a 32-year-old shop manager, presents with detachment from social relationships and restricted emotional expression with little desire for close relationships. Which diagnosis or clinical concept best fits this presentation?

A. Obsessive-compulsive personality disorder (OCPD)

B. Stimulant intoxication

C. Bipolar disorder

**D. Schizoid personality disorder**

**Answer: D - Schizoid personality disorder.** The key pattern is social detachment and limited affect, rather than fear of rejection as in avoidant personality.

**Memory aid:** SCHIZOID = Solitary by preference.

7. During assessment, Daniel reports excessive need to be cared for, difficulty making decisions independently, submissiveness and fear of separation. Which of the following is the BEST match?

A. Obsessive-compulsive disorder (OCD)

B. Opioid intoxication

**C. Dependent personality disorder**

D. Manic episode

**Answer: C - Dependent personality disorder.** Dependent personality is characterized by reliance on others for decisions, reassurance and support.

**Memory aid:** DEPENDENT = Decisions Entrusted to People; separation fear.

8. Joel, a 44-year-old farmer, presents with persistent social-communication difficulties together with restricted or repetitive patterns of behavior or interests beginning in development. Which diagnosis or clinical concept best fits this presentation?

A. Schizophrenia

B. Obsessive-compulsive disorder (OCD)

**C. Autism spectrum disorder**

D. Stimulant intoxication

**Answer: C - Autism spectrum disorder.** Autism requires both social-communication differences and restricted/repetitive behavior patterns.

**Memory aid:** 2R domains: Relating/communication + Repetitive/restricted behavior.

9. During assessment, Amina reports recurrent unexpected panic attacks followed by persistent worry about more attacks or maladaptive avoidance. Which of the following is the BEST match?

A. Obsessive-compulsive disorder (OCD)

B. Post-traumatic stress disorder (PTSD)

C. Bulimia nervosa

#### D. Panic disorder

**Answer: D - Panic disorder.** Panic disorder is more than a single panic attack: recurrent unexpected attacks are followed by ongoing concern or behavior change.

**Memory aid:** PANIC: Panic Attacks + New worry/avoidance Is Continuing.

**10.** A therapist notes that Owen has excessive, difficult-to-control worry across several life areas for about six months, with tension, restlessness or poor sleep. Which condition should be considered first from the options below?

- A. Agoraphobia
- B. Brief psychotic disorder
- C. Generalized anxiety disorder (GAD)**
- D. Anorexia nervosa, restricting type

**Answer: C - Generalized anxiety disorder (GAD).** GAD is characterized by persistent generalized worry across multiple domains rather than fear tied to one object or situation.

**Memory aid:** GAD = General Anxiety across Domains.

#### SECTION B: TRUE/FALSE - EXPLANATIONS

**1. TRUE.** A panic attack can occur in disorders other than panic disorder.

A panic attack is a symptom episode and can occur across several disorders; panic disorder requires a recurrent pattern plus ongoing concern or behavior change.

**2. FALSE.** Obsessions are repetitive behaviors, whereas compulsions are intrusive thoughts.

The relationship is reversed: obsessions are intrusive thoughts/urges/images; compulsions are repetitive behaviors or mental acts.

**3. TRUE.** Trauma exposure can increase risk for depression, anxiety and substance-related problems.

Trauma can be followed by several forms of psychopathology, not only PTSD.

**4. FALSE.** A single laboratory test confirms schizophrenia.

Schizophrenia is diagnosed clinically from symptoms, course, duration and impairment while ruling out other causes.

**5. TRUE.** In major depression, loss of interest or pleasure is called anhedonia.

Anhedonia is a core depressive symptom.

#### SECTION C: MODEL SHORT ANSWERS

**1. Differentiate anorexia nervosa from bulimia nervosa. (2mks)**

- Anorexia requires significantly low body weight with restriction/fear of weight gain/body-image disturbance.
- Bulimia centers on recurrent binge eating plus compensatory behavior; markedly low weight is not required.

**Memory aid:** Low weight points strongly toward anorexia.

**2. State two characteristic features of a manic episode. (2mks)**

- Reduced need for sleep, grandiosity, pressured speech, flight of ideas, increased goal-directed activity, risky behavior - any two.

**Memory aid:** DIG FAST.

**3. What is comorbidity? (2mks)**

- The presence of two or more disorders in the same person at the same time or over an overlapping period.

**Memory aid:** CO-morbidity = conditions CO-occur.

**4. Name the four broad PTSD symptom clusters taught in modern formulations. (2mks)**

- Intrusion
- Avoidance
- Negative changes in cognition/mood
- Arousal/reactivity

**Memory aid:** IANA.

**5. What is meant by anhedonia? (2mks)**

- Marked loss of interest or pleasure in activities previously enjoyed; a core depressive symptom.

**Memory aid:** *AN-hedonia* = absence of pleasure.

## PART TWO: THEORIES/TECHNIQUES - ANSWERS

1. A therapist working with Carlos uses an intervention characterized by the client unconsciously avoids material or processes that threaten to bring unacceptable conflict into awareness. Which approach or technique is being used?

- A. Gestalt therapy
- B. Flooding
- C. Resistance**
- D. Systematic desensitization

**Answer: C - Resistance.** Resistance is the defensive blocking of threatening material or therapeutic work.

**Memory aid:** *Resistance = psychological brakes.*

2. A therapist working with Zachary uses an intervention characterized by the client redirects feelings, expectations or relationship patterns from important past relationships onto the therapist. Which approach or technique is being used?

- A. Interpretation
- B. Transference**
- C. Empty-chair technique
- D. Rational Emotive Behavior Therapy (REBT)

**Answer: B - Transference.** Transference originates primarily from the client's earlier relationships and becomes enacted in the therapeutic relationship.

**Memory aid:** *TRANSference = client TRANSfers old relationship feelings to therapist.*

3. In a session with Martha, the counselor emphasizes the therapist's emotional reactions to a client, especially reactions influenced by the therapist's own history or unresolved issues. This BEST illustrates:

- A. Flooding
- B. In vivo exposure
- C. Retroflexion
- D. Countertransference**

**Answer: D - Countertransference.** Countertransference concerns the therapist's responses to the client and must be monitored through self-awareness and supervision.

**Memory aid:** *COUNTER = counselor side of the relational reaction.*

4. In a session with Irene, the counselor emphasizes therapy emphasizing unconscious conflict, early relationships, defenses, transference, resistance and insight. This BEST illustrates:

- A. Psychoanalytic therapy**
- B. Gestalt dream work
- C. Extinction
- D. Flooding

**Answer: A - Psychoanalytic therapy.** Psychoanalytic approaches seek insight into unconscious patterns and how early relationships repeat in present life and therapy.

**Memory aid:** *FIRT: Free association, Interpretation, Resistance, Transference.*

5. A therapist working with Lucas uses an intervention characterized by a goal-directed, socially embedded approach emphasizing conscious choice, responsibility, lifestyle and movement from inferiority toward useful belonging. Which approach or technique is being used?

- A. Operant conditioning
- B. Free association
- C. Gestalt therapy
- D. Adlerian therapy**

**Answer: D - Adlerian therapy.** Adlerian therapy views behavior as purposeful and socially related, with emphasis on goals, choice and responsibility. This is explicitly reflected in the opening lecture of the fused audio.

**Memory aid:** *GSCR: Goal-directed, Social relatedness, Conscious choice, Responsibility.*

6. In a session with Victor, the counselor emphasizes a non-directive relationship based on accurate empathy, unconditional positive regard and congruence. This BEST illustrates:

- A. Cognitive restructuring
- B. Exposure therapy
- C. Person-centered therapy**
- D. Transference

**Answer: C - Person-centered therapy.** Rogers' person-centered therapy relies on core facilitative conditions rather than therapist-imposed solutions.

**Memory aid:** EUC: Empathy, Unconditional positive regard, Congruence.

7. A therapist working with Victor uses an intervention characterized by a structured approach linking thoughts, emotions and behaviors and using cognitive and behavioral change strategies. Which approach or technique is being used?

- A. Resistance
- B. Flooding
- C. Rational emotive imagery
- D. Cognitive behavioral therapy (CBT)**

**Answer: D - Cognitive behavioral therapy (CBT).** CBT targets maladaptive thinking and behavior through collaborative, structured methods such as cognitive restructuring and behavioral experiments.

**Memory aid:** TEA: Thoughts - Emotions - Actions.

8. In a session with Yvette, the counselor emphasizes identifying inaccurate or dysfunctional thoughts, examining evidence and developing more balanced alternatives. This BEST illustrates:

- A. Person-centered therapy
- B. Systematic desensitization
- C. Rational Emotive Behavior Therapy (REBT)
- D. Cognitive restructuring**

**Answer: D - Cognitive restructuring.** Cognitive restructuring is a core CBT method for testing and modifying distorted or unhelpful cognitions.

**Memory aid:** Catch it, Check it, Change it.

9. A therapist working with Tina uses an intervention characterized by Albert Ellis' approach that identifies irrational beliefs between events and emotional consequences and actively disputes those beliefs. Which approach or technique is being used?

- A. Gestalt therapy
- B. Empty-chair technique
- C. Rational Emotive Behavior Therapy (REBT)**
- D. Retroflexion

**Answer: C - Rational Emotive Behavior Therapy (REBT).** REBT uses the ABC model and disputation to replace rigid irrational beliefs with more rational alternatives.

**Memory aid:** ABCDE: Activating event, Belief, Consequence, Dispute, Effective new belief.

10. Which psychotherapy concept is most accurately described by the following: an REBT imagery exercise in which the client vividly imagines a difficult event, experiences the emotion, then practices changing unhealthy emotion by changing beliefs?

- A. Rational emotive imagery**
- B. In vivo exposure
- C. Rational Emotive Behavior Therapy (REBT)
- D. Social skills training

**Answer: A - Rational emotive imagery.** Rational emotive imagery is an experiential REBT technique for rehearsing healthier emotional responses through belief change.

**Memory aid:** Imagine - feel - dispute - re-feel.

11. A therapist working with Rose uses an intervention characterized by a here-and-now experiential approach emphasizing awareness, contact, unfinished business and integration. Which approach or technique is being used?

- A. Cognitive behavioral therapy (CBT)
- B. Resistance
- C. Systematic desensitization

**D. Gestalt therapy**

**Answer: D - Gestalt therapy.** Gestalt therapy focuses on present awareness and experiments that help clients own and integrate experience.

**Memory aid:** *Gestalt = awareness in the NOW.*

12. In a session with Grace, the counselor emphasizes the client speaks to an imagined person or part of self placed in an empty chair, often switching roles to deepen awareness. This BEST illustrates:

- A. Countertransference
- B. Empty-chair technique**
- C. Operant conditioning
- D. In vivo exposure

**Answer: B - Empty-chair technique.** The empty-chair experiment is associated with Gestalt work on unfinished business, polarities and present-moment experience.

**Memory aid:** *Empty chair = make the absent relationship present.*

13. Which psychotherapy concept is most accurately described by the following: the client re-enacts a dream in the present and may speak as different people, objects or elements of the dream?

- A. Exposure therapy
- B. Cognitive behavioral therapy (CBT)
- C. Gestalt dream work**
- D. Transference

**Answer: C - Gestalt dream work.** Rather than imposing a fixed interpretation, Gestalt dream work invites the client to own and experience parts of the dream.

**Memory aid:** *Dream it, become it, speak it.*

14. Which psychotherapy concept is most accurately described by the following: turning back onto oneself what one wants to do to another, or doing to oneself what one wishes another would do?

- A. Retroflexion**
- B. Social skills training
- C. Person-centered therapy
- D. Classical conditioning

**Answer: A - Retroflexion.** Retroflexion is a Gestalt contact-boundary disturbance directed back toward the self.

**Memory aid:** *RETRO = return action back to self.*

15. In a session with Uma, the counselor emphasizes an approach emphasizing freedom, responsibility, choice, meaning, authenticity, isolation and awareness of mortality. This BEST illustrates:

- A. Gestalt dream work
- B. Progressive muscle relaxation
- C. Existential therapy**
- D. Systematic desensitization

**Answer: C - Existential therapy.** Existential therapy helps clients face givens of existence and make authentic choices rather than simply trying to eliminate all anxiety.

**Memory aid:** *FMRC: Freedom, Meaning, Responsibility, Choice.*

## SECTION B: TRUE/FALSE - EXPLANATIONS

1. **TRUE.** Accurate empathy, unconditional positive regard and congruence are core person-centered conditions.

These are the classic Rogers conditions emphasized in person-centered therapy.

**2. TRUE.** Albert Ellis is the major proponent of Rational Emotive Behavior Therapy.

Ellis developed REBT.

**3. TRUE.** Sigmund Freud is the foundational proponent of psychoanalytic therapy.

Freud developed psychoanalysis.

**4. FALSE.** Free association requires the therapist to censor embarrassing client material.

The client is invited to say what comes to mind without censorship.

**5. FALSE.** Transference refers mainly to the therapist transferring personal feelings onto the client.

That description is closer to countertransference; transference is the client carrying past relationship patterns into the therapeutic relationship.

### **SECTION C: MODEL SHORT ANSWERS**

**1. What is cognitive restructuring? (2mks)**

- Identify unhelpful/biased thoughts, examine evidence, and generate more balanced alternatives, then test them behaviorally when appropriate.

**Memory aid:** *Catch - Check - Change.*

**2. State two principles emphasized in Adlerian therapy. (2mks)**

- Any two: behavior is purposeful/goal-directed; social relatedness/social interest; conscious choice; responsibility; lifestyle; encouragement.

**Memory aid:** *GSCR.*

**3. What does extinction mean in behavior therapy? (1mks)**

- A reinforced behavior decreases when the reinforcement maintaining it is consistently withheld.

**Memory aid:** *Extinguish the payoff.*

### PART THREE: ETHICAL/LEGAL - ANSWERS

1. During supervision, Wendy describes taking reasonable protective action when a client presents a serious, credible threat toward an identifiable person, as required by applicable law and ethics. Which concept should guide the discussion?

- A. Veracity
- B. Duty to warn/protect**
- C. Privacy
- D. Professional competence

**Answer: B - Duty to warn/protect.** The course tests duty to warn/protect as an exception to confidentiality when serious danger to others is present. Actual legal requirements vary by jurisdiction.

**Memory aid:** *TIP: Threat serious, Identifiable target, Protect reasonably.*

2. A counselor asks which principle or professional concept is defined by reporting suspected abuse or other legally reportable situations to designated authorities when law or professional duty requires it. The BEST answer is:

- A. Autonomy
- B. Ethical response to colleague misconduct
- C. Mandated reporting**
- D. Confidentiality

**Answer: C - Mandated reporting.** Mandated reporting is a recognized limit to confidentiality; therapists should explain this limit during informed consent.

**Memory aid:** *Report when law makes reporting mandatory.*

3. In professional practice, a situation calls for professional negligence in which a duty is breached and the breach causes harm. Which ethical/legal concept is MOST directly involved?

- A. Ethical response to colleague misconduct
- B. Malpractice**
- C. Veracity
- D. Record keeping

**Answer: B - Malpractice.** Malpractice requires more than an unfavorable outcome: there must be a professional duty, breach, harm and causal link.

**Memory aid:** *DBHC: Duty, Breach, Harm, Causation.*

4. In professional practice, a situation calls for practicing only within areas supported by appropriate education, training, supervised experience and current skill. Which ethical/legal concept is MOST directly involved?

- A. Professional competence**
- B. Privileged communication
- C. Record keeping
- D. Duty to warn/protect

**Answer: A - Professional competence.** Using techniques without adequate training can expose clients to harm and the professional to ethical or legal liability.

**Memory aid:** *Know your scope; seek supervision or refer.*

5. During supervision, Zachary describes a situation in which the therapist has another significant role or relationship with the client in addition to therapy. Which concept should guide the discussion?

- A. Autonomy
- B. Subpoena response
- C. Dual or multiple relationship**
- D. Malpractice

**Answer: C - Dual or multiple relationship.** Multiple relationships are not automatically harmful in every context, but should be avoided or carefully managed when they risk exploitation, impaired objectivity or harm.

**Memory aid:** *Two roles = review power, objectivity, exploitation and harm.*

6. A counselor asks which principle or professional concept is defined by a departure from customary therapeutic boundaries that may be clinically appropriate and non-exploitative in context. The BEST answer is:

- A. Boundary crossing**
- B. Duty to warn/protect
- C. Termination and referral
- D. Bartering

**Answer: A - Boundary crossing.** A boundary crossing is not automatically unethical; context, culture, intent and effect matter.

**Memory aid:** Crossing may be thoughtful; violation causes or risks serious harm/exploitation.

7. A counselor asks which principle or professional concept is defined by a serious harmful or exploitative breach of therapeutic boundaries. The BEST answer is:

- A. Fidelity
- B. Boundary violation**
- C. Justice
- D. Gift decision-making

**Answer: B - Boundary violation.** Boundary violations exploit the therapeutic relationship or cause significant harm and are unethical.

**Memory aid:** Violation = harm/exploitation.

8. A counselor asks which principle or professional concept is defined by sexual or romantic involvement with a current therapy client. The BEST answer is:

- A. Sexual relationship with a current client**
- B. Termination and referral
- C. Informed consent
- D. Nonmaleficence

**Answer: A - Sexual relationship with a current client.** Sexual relationships with current clients are prohibited because of power imbalance and exploitation risk. Terminating therapy to start a relationship does not erase those concerns.

**Memory aid:** Current client + sex = no.

9. In professional practice, a situation calls for considering clinical meaning, value, timing, culture, power and exploitation risk before accepting or declining a client gift. Which ethical/legal concept is MOST directly involved?

- A. Technology ethics
- B. Gift decision-making**
- C. Veracity
- D. Ethical response to colleague misconduct

**Answer: B - Gift decision-making.** Gifts require contextual judgment. A culturally meaningful modest gift may be handled differently from an expensive or coercive gift.

**Memory aid:** VALUE: Value, Understanding/meaning, Local culture, Unequal power, Effect on therapy.

10. In professional practice, a situation calls for accepting goods or services instead of money when ethically permissible, not exploitative and not clinically contraindicated. Which ethical/legal concept is MOST directly involved?

- A. Bartering**
- B. Veracity
- C. Malpractice
- D. Gift decision-making

**Answer: A - Bartering.** Bartering may be acceptable under some codes when not exploitative and when arrangements are clear and clinically appropriate.

**Memory aid:** Barter only if Boundaries and Benefits are safe.

11. In professional practice, a situation calls for maintaining accurate, timely, secure and clinically appropriate client records. Which ethical/legal concept is MOST directly involved?

- A. Record keeping**

- B. Ethical response to colleague misconduct
- C. Boundary crossing
- D. Fidelity

**Answer: A - Record keeping.** Records support continuity of care, accountability and legal/ethical duties; access must be controlled and retention rules followed.

**Memory aid:** ACTS: Accurate, Current, Timely, Secure.

**12.** During supervision, Fidel describes ending therapy responsibly with preparation, continuity planning and referral when needed rather than abandoning the client. Which concept should guide the discussion?

- A. Termination and referral**
- B. Confidentiality
- C. Duty to warn/protect
- D. Ethical response to colleague misconduct

**Answer: A - Termination and referral.** Ethical termination includes adequate notice, discussion, referrals or continuity plans when appropriate.

**Memory aid:** PLAN: Prepare, Link/referral, Address feelings, Next steps.

**13.** In professional practice, a situation calls for using ethical codes, consultation/supervision and appropriate reporting procedures when credible serious misconduct is observed. Which ethical/legal concept is MOST directly involved?

- A. Confidentiality
- B. Ethical response to colleague misconduct**
- C. Bartering
- D. Gift decision-making

**Answer: B - Ethical response to colleague misconduct.** Serious ethical violations should not simply be ignored; follow code, organizational and legal procedures while protecting clients and due process.

**Memory aid:** C-A-R-E: Consult code, Assess facts, Report appropriately, Ensure client safety.

**14.** In professional practice, a situation calls for protecting privacy, competence, informed consent, identity, emergency planning and data security when using digital or remote services. Which ethical/legal concept is MOST directly involved?

- A. Technology ethics**
- B. Mandated reporting
- C. Confidentiality
- D. Malpractice

**Answer: A - Technology ethics.** Technology changes the medium, not the professional duty. Security, consent and contingency planning remain essential.

**Memory aid:** SPACE: Security, Privacy, Access/identity, Consent, Emergency plan.

**15.** During supervision, Joel describes not automatically disclosing the entire file on receipt of a subpoena; consult applicable law, the client/attorney and professional guidance, and disclose only as authorized or legally required. Which concept should guide the discussion?

- A. Confidentiality
- B. Informed consent
- C. Record keeping
- D. Subpoena response**

**Answer: D - Subpoena response.** A subpoena and a court order are not always identical. The safest course-based principle is not to reflexively release all records; seek proper legal/ethical guidance and protect confidentiality to the extent permitted.

**Memory aid:** STOP: See legal guidance, Tell/consult client, Only required information, Protect confidentiality.

## SECTION B: MODEL SHORT ANSWERS

**1.** How should a therapist respond to a subpoena for records? (2mks)

- Do not reflexively release the entire file.

- Consult applicable law/professional guidance and the client/attorney; protect confidentiality and disclose only what is authorized or legally required.

**Memory aid:** STOP.

**2. What is the principle of justice? (1mks)**

- Fair and equitable treatment and distribution of benefits, burdens and resources.

**Memory aid:** Justice = fairness.

**3. What does duty to warn/protect mean in therapy? (1mks)**

- When a serious credible threat to an identifiable person is present, the therapist may have to take reasonable protective action according to applicable law and ethics, even though confidentiality is normally protected.

**Memory aid:** TIP: Threat, Identifiable person, Protect.

**4. Define mandated reporting. (1mks)**

- A legal/professional requirement to report specified situations such as suspected abuse to designated authorities; it is a limit to confidentiality.

**Memory aid:** Mandatory = must report when the law says so.

**SECTION C: MODEL SHORT ESSAYS**

**1. State two ways a therapist should manage sexual or romantic feelings toward a client. (2mks)**

- Acknowledge and monitor the feelings rather than act on them.

- Seek supervision/consultation and strengthen boundaries; refer if objectivity/competence cannot be maintained.

**Memory aid:** FEEL, do not ACT: notice, supervise, protect boundaries.

**2. What should a therapist do when credible serious unethical behavior by a colleague is suspected? (2mks)**

- Clarify facts, consult the code/supervisor as appropriate, protect the client and follow formal reporting procedures when required.

**Memory aid:** CARE: Consult, Assess, Report, Ensure safety.

**3. Differentiate privacy from confidentiality. (1mks)**

- Privacy is the client's right to control access to self and personal information.

- Confidentiality is the professional duty to protect information obtained in the professional relationship.

**Memory aid:** Privacy = person's right; confidentiality = professional's duty.

## PART FOUR: CULTURE / GROUP / COUNSELING PROCESS / TREATMENT PLAN - ANSWERS

1. In counseling practice, Patricia encounters a situation involving pressuring a client to adopt the therapist's or dominant culture's beliefs, values or practices. Which concept BEST applies?

**A. Cultural imposition**

- B. Assessment
- C. Reflection of feeling
- D. Intervention stage

**Answer: A - Cultural imposition.** Cultural imposition occurs when the helper pushes their own values or practices onto the client.

**Memory aid:** IMPOSITION = impose my culture.

2. Which counseling/group/cultural concept is BEST described by: using a qualified interpreter rather than relying on a child or untrained friend when language barriers affect care?

- A. Open-ended questioning

**B. Professional interpreter use**

- C. Treatment plan
- D. Cultural competence

**Answer: B - Professional interpreter use.** Qualified interpreters improve accuracy, confidentiality and informed consent.

**Memory aid:** Interpreter = accuracy + confidentiality.

3. In counseling practice, Zachary encounters a situation involving a group emphasizing mutual support, shared experience, acceptance and coping among people facing similar problems. Which concept BEST applies?

- A. Therapeutic confrontation

**B. Support group**

- C. Treatment plan
- D. Norming stage

**Answer: B - Support group.** Support groups reduce isolation and offer mutual aid through shared experience.

**Memory aid:** SUPPORT = Similar people Understanding and Providing Peer help.

4. A trainee observes a structured group focused on teaching information, understanding a condition and coping knowledge. This is an example of:

- A. Ethnocentrism
- B. Active listening

**C. Psychoeducational group**

- D. Reflection of feeling

**Answer: C - Psychoeducational group.** Psychoeducational groups primarily teach information and understanding; they may include discussion and practice.

**Memory aid:** EDU = education is central.

5. In counseling practice, Lucas encounters a situation involving a group focused on learning and practicing specific behavioral or interpersonal skills. Which concept BEST applies?

- A. Intervention stage

**B. Skills-development group**

- C. Ethnocentrism
- D. Self-disclosure

**Answer: B - Skills-development group.** Skills groups emphasize rehearsal, practice and feedback rather than information alone.

**Memory aid:** SKILLS = Show, Keep practicing, Improve, Learn, feedback.

6. In counseling practice, Joel encounters a situation involving a structured group using cognitive and behavioral methods such as thought monitoring, skills practice and behavioral exercises. Which concept BEST applies?

**A. Cognitive-behavioral group**

- B. Storming stage
- C. Progress review
- D. Support group

**Answer: A - Cognitive-behavioral group.** CBT groups combine structured learning with practice and feedback around thoughts and behaviors.

**Memory aid:** Group CBT = Think + Do together.

7. Which counseling/group/cultural concept is BEST described by: a recurrent functional position adopted by a member, such as scapegoat, monopolizer, peacemaker or spokesperson?

- A. Treatment plan
- B. Group role**
- C. Storming stage
- D. Open-ended questioning

**Answer: B - Group role.** Group roles describe patterned ways members participate and influence group dynamics.

**Memory aid:** Role = repeated position in the group.

8. In counseling practice, Nadine encounters a situation involving an agreement that members protect information disclosed by other members, while the leader explains that peer confidentiality cannot be guaranteed in the same way as professional confidentiality. Which concept BEST applies?

- A. Cultural competence
- B. Performing stage
- C. Assessment
- D. Group confidentiality**

**Answer: D - Group confidentiality.** Confidentiality is essential in groups, but leaders should explain realistic limits because members are not all licensed professionals.

**Memory aid:** What is said in group should stay in group - with limits explained.

9. Which counseling/group/cultural concept is BEST described by: early group stage characterized by orientation, uncertainty, dependence and testing?

- A. Active listening
- B. Cultural imposition
- C. Open-ended questioning
- D. Forming stage**

**Answer: D - Forming stage.** Forming is the beginning stage in which members learn the purpose, structure and safety of the group.

**Memory aid:** FSNPA: Forming, Storming, Norming, Performing, Adjourning.

10. A trainee observes group stage in which conflict, resistance, competition or emotional testing becomes more visible. This is an example of:

- A. Assessment
- B. Psychoeducational group
- C. Active listening
- D. Storming stage**

**Answer: D - Storming stage.** Storming often brings disagreement and challenges to roles or leadership.

**Memory aid:** Storm = conflict weather.

11. Which counseling/group/cultural concept is BEST described by: group stage in which cohesion, trust, norms and cooperation consolidate?

- A. Norming stage**
- B. Adjourning stage
- C. Goal setting

D. Paraphrasing

**Answer: A - Norming stage.** Norming is marked by clearer expectations and stronger cohesion.

**Memory aid:** *Norming = norms settle.*

12. A trainee observes mature working stage in which members use the group productively toward goals. This is an example of:

**A. Performing stage**

B. Open-ended questioning

C. Storming stage

D. Rapport building

**Answer: A - Performing stage.** Performing is the task-focused, therapeutically productive phase.

**Memory aid:** *Performing = productive work.*

13. Which counseling/group/cultural concept is BEST described by: ending stage involving review, separation, consolidation and transfer of learning?

A. Therapeutic confrontation

B. Open-ended questioning

C. Norming stage

**D. Adjourning stage**

**Answer: D - Adjourning stage.** Adjourning addresses termination and helps members carry gains beyond the group.

**Memory aid:** *Adjourning = goodbye and generalize gains.*

14. A trainee observes creating safety, trust, empathy and collaborative engagement at the beginning of counseling. This is an example of:

A. Professional interpreter use

B. Cultural competence

C. Case formulation

**D. Rapport building**

**Answer: D - Rapport building.** Rapport supports disclosure and collaboration; it begins early but continues throughout treatment.

**Memory aid:** *RAPPORT starts with Respect, Attention, Presence.*

15. A trainee observes systematically exploring the client's problems, symptoms, context, history, strengths, risk and functioning. This is an example of:

A. Case formulation

**B. Assessment**

C. Group confidentiality

D. Goal setting

**Answer: B - Assessment.** Assessment precedes or accompanies formulation and goal setting; it includes both problems and resources.

**Memory aid:** *HRSF: History, Risk, Symptoms, Functioning.*

## SECTION B: MODEL SHORT ESSAYS

1. Outline two benefits of group therapy. **(4mks)**

- Reduces isolation/universality: members learn they are not alone.
- Provides mutual support and feedback.
- Allows interpersonal learning, modeling and practice.
- Can be efficient and increase access - any two well explained.

**Memory aid:** *U-S-M: Universality, Support, Modeling.*

2. State two characteristics of a culturally competent therapist. **(4mks)**

- Self-awareness of own values/biases.
- Knowledge of different cultural contexts and humility about what is not known.

- Adaptable communication/clinical skills.
- Willingness to seek cultural encounters/consultation and avoid imposition - any two explained.

**Memory aid:** AKSE.

**3. What is the counseling process? (2mks)**

- A collaborative sequence that typically includes engagement/rapport, assessment, goal setting, intervention, review, and termination/follow-up.

**Memory aid:** RAGIRT: Rapport, Assessment, Goals, Intervention, Review, Termination.

## SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

|                                      |                                                       |
|--------------------------------------|-------------------------------------------------------|
| <b>Session:</b> Revision Mock Set 05 | <b>Duration:</b> 1hr 30mins                           |
| <b>Specialty:</b> Psychotherapy      | <b>Coefficient:</b> 04                                |
| <b>Eliminatory mark:</b> < 14/20     | <b>Training reference:</b> VTI-AEDA examination style |

## SIMULATION OF REAL WORKING SITUATION PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 05

*Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.*

### DETAILED MODEL ANSWERS - SIMULATION OF REAL WORKING SITUATIONS

#### SECTION I

##### 1. Cultural conflict in counseling - anchor: Culturally responsive person-centered counseling

Helen, 47, is torn between personal career/relationship goals and strong expectations from family and community. A previous helper told her, "You need to stop living by old traditions." Helen felt judged and stopped counseling.

##### **Model response / marking points (25 marks):**

1. Acknowledge the prior experience of judgment and rebuild safety.
2. Practice cultural humility; ask rather than assume what family/community values mean to this client.
3. Explore both individual and relational/community values.
4. Avoid cultural imposition and ethnocentrism.
5. Use empathy, reflection and open questions.
6. Assess the client's own goals rather than imposing individualistic independence.
7. Explore possible options and consequences in the client's context.
8. Identify supports and cultural strengths.
9. Collaboratively set goals consistent with the client's values.
10. Reflect on therapist bias and seek supervision/consultation if needed.

**Suggested mark allocation:** Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

**Memory aid:** AKSE: Awareness, Knowledge, Skills, Encounters; ask, do not impose.

##### 2. Trauma-related guilt and cognitive restructuring - anchor: Trauma-focused CBT principles with cognitive restructuring and graded exposure when ready

Ibrahim, 54, survived an accident in which another person was injured. Although investigators found Ibrahim was not at fault, he repeatedly thinks, "If I had reacted one second faster none of this would have happened. It was my fault." Avoidance and nightmares continue.

##### **Model response / marking points (25 marks):**

1. Assess PTSD symptoms, dissociation, safety and current functioning.
2. Identify hindsight bias, inflated responsibility and counterfactual thinking.
3. Examine evidence for and against personal responsibility using what was knowable at the time.
4. Develop a balanced statement acknowledging regret without false blame.
5. Use behavioral reduction of avoidance and trauma-focused exposure only with consent/readiness.
6. Teach grounding for high arousal/dissociation.

7. Address meaning and self-forgiveness carefully.
8. Use homework such as thought records.
9. Monitor nightmares/avoidance and risk.
10. Coordinate other care if symptoms are severe.

**Suggested mark allocation:** Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

**Memory aid:** *Responsibility check: Control? Knowledge at the time? Intent? Realistic alternatives?*

## SECTION II

### 1. Perfectionism and overcontrol - anchor: OCPD features - CBT/personality-focused assessment

Janet, 61, spends so much time checking and perfecting work that deadlines are missed. At home she insists family members follow detailed schedules and becomes upset when others do tasks differently. Janet sees these standards as necessary and correct.

#### **Model response / marking points (25 marks):**

1. Identify pervasive perfectionism, rigidity and control consistent with OCPD features.
2. Differentiate from OCD by asking about intrusive obsessions and rituals; OCPD can be ego-syntonic standards rather than unwanted obsessions.
3. Assess impact across work, relationships and flexibility.
4. Explore costs and benefits of rules/perfectionism.
5. Identify all-or-nothing beliefs and fear of mistakes.
6. Use behavioral experiments with "good enough" performance and delegation.
7. Build cognitive flexibility and collaborative goals.
8. Address interpersonal impact and empathy.
9. Avoid power struggles over control.
10. Review progress using functional outcomes, not just symptom labels.

**Suggested mark allocation:** Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

**Memory aid:** *OCPD = Order, Control, Perfectionism, Duty.*

### 2. Treatment planning after assessment - anchor: Collaborative treatment planning

Following assessment, Kevin, 20, reports depressed mood, social withdrawal, irregular sleep and work absence. No imminent suicide plan is found, but functioning is poor. Kevin says the main wish is "to get back to a normal routine and feel useful again."

#### **Model response / marking points (25 marks):**

1. Define problems in observable, client-centered language (depressed mood, withdrawal, sleep disruption, work impairment).
2. Write a broad client goal linked to the client's wish.
3. Create SMART objectives, e.g. two scheduled activities on four days/week; consistent wake time; one supportive contact/week.
4. Select interventions linked to formulation: behavioral activation, CBT thought work, sleep routine, support building.
5. Include ongoing risk monitoring.
6. Specify review date/criteria and responsibility for homework.
7. Include referral/medical review if indicated.
8. Document strengths/protective factors.
9. Ensure plan is collaborative and culturally realistic.
10. Revise according to measured progress.

**Suggested mark allocation:** Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

**Memory aid:** *PGOIR: Problem, Goal, Objectives, Interventions, Review.*